(Link: www.nhs.uk/)

Mouth ulcers

Mouth ulcers are common and should clear up on their own within a week or 2. But see a GP or dentist if you have a mouth ulcer that lasts longer than 3 weeks.

How you can treat mouth ulcers yourself

Mouth ulcers are rarely a sign of anything serious, but may be uncomfortable to live with.

They need time to heal and there's no quick fix.

Avoiding things that irritate your mouth ulcer should help:

- speed up the healing process
- reduce pain
- reduce the chance of it returning

Do

- ✓ use a soft-bristled toothbrush
- ✓ drink cool drinks through a straw
- ✓ eat softer foods
- ✓ get regular dental check-ups
- ✓ eat a healthy, balanced diet

Don’t

- ✗ do not eat very spicy, salty or acidic food
- ✗ do not eat rough, crunchy food, such as toast or crisps
- ✗ do not drink very hot or acidic drinks, such as fruit juice
- ✗ do not use toothpaste containing sodium lauryl sulphate

A pharmacist can help with mouth ulcers

A pharmacist can recommend a treatment to speed up healing, prevent infection or reduce pain, such as:

- antimicrobial mouthwash
- a painkilling tablet, mouthwash, gel or spray
- corticosteroid lozenges
- a salt (saline) mouthwash

You can buy these without a prescription.

▼ How to rinse with salt mouthwash

1. Dissolve half a teaspoon of salt in a glass of warm water – warm water helps salt dissolve.
2. Rinse your mouth with the solution, then spit it out – do not swallow it.
3. Repeat as often as you like.

Find a pharmacy (Link: https://www.nhs.uk/service-search/pharmacy/find-a-pharmacy)

See a dentist or GP if your mouth ulcer:

- lasts longer than 3 weeks
- is different to other mouth ulcers you've had before, for example if it's bigger than usual or near the back of your throat
- bleeds or becomes more painful and red – this may be a sign of an infection

Although most mouth ulcers are harmless, a long-lasting mouth ulcer is sometimes a sign of mouth cancer. It's best to get it checked.

You should also see a GP if you have other symptoms such as:

- ulcers anywhere else on your body, such as your skin or genitals
- painful, red or swollen joints

Treatment from a dentist or GP

A GP or dentist may prescribe stronger medicine to treat severe, persistent or infected mouth ulcers.

Possible treatments include:

- steroids given as either a mouth spray, tablets that dissolve in your mouth or tablets that can be dissolved in water
- painkilling gels, ointments, sprays or tablets
- mouthwashes to kill or remove any germs in your mouth

Check if you have a mouth ulcer



Mouth ulcers usually appear inside the mouth on the cheeks or lips.



Ulcers can also appear on the tongue.

You may have more than 1 ulcer at a time.

Mouth ulcers are not contagious and should not be confused with cold sores (Link: www.nhs.uk/conditions/cold-sores/).

Cold sores appear on the lips or around the mouth and often begin with a tingling, itching or burning sensation. They can also appear as a small group of pinhead-sized ulcers inside the mouth.

Causes of mouth ulcers

Most single mouth ulcers are caused by things you can try to avoid, such as:

- biting the inside of your cheek
- badly fitting dentures, braces, rough fillings or a sharp tooth
- cuts or burns while eating or drinking – for example, hard food or hot drinks
- a food intolerance or allergy
- damaging your gums with a toothbrush or irritating toothpaste
- feeling tired, stressed or anxious

Sometimes they're triggered by things you cannot always control, such as:

- hormonal changes – such as during pregnancy
- your genes – some families get mouth ulcers more often
- a vitamin deficiency, such as iron, zinc, folic acid, vitamin B or vitamin D
- medicines – including some NSAIDs, beta blockers or nicorandil
- stopping smoking – people may develop mouth ulcers when they first stop smoking

If you have several mouth ulcers, it can be a symptom of:

- hand, foot and mouth disease (Link: www.nhs.uk/conditions/hand-foot-mouth-disease/), which also causes a rash on the hands and feet
- oral lichen planus (Link: www.nhs.uk/conditions/lichen-planus/), which causes a white, lacy pattern inside the cheeks
- Crohn's disease (Link: www.nhs.uk/conditions/crohns-disease/) and coeliac disease (Link: www.nhs.uk/conditions/coeliac-disease/) (conditions that affect the digestive system)
- a weakened immune system from having a condition like HIV (Link: www.nhs.uk/conditions/hiv-and-aids/) or lupus (Link: www.nhs.uk/conditions/lupus/)

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